



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK-3 PAPER-2 DATED ON 26th MAY 2026

QUESTIONS

Q.1) A 34-year-old woman with autoimmune hypothyroidism presents with vomiting, abdominal pain, severe weakness, and confusion after a febrile illness. BP is 78/46 mmHg, pulse 126/min. Skin appears diffusely hyperpigmented, especially over palmar creases and buccal mucosa. Labs show Na^+ 122 mEq/L, K^+ 6.1 mEq/L, glucose 52 mg/dL. Which is the best immediate treatment?

- a) Levothyroxine
- b) Hydrocortisone with IV normal saline
- c) Fludrocortisone alone
- d) Propranolol

Q.2) A 23-year-old man has hemoptysis, rapidly progressive renal failure, and hematuria. Renal biopsy shows crescentic glomerulonephritis with linear IgG deposition. Target antigen?

- a) Type IV collagen
- b) Desmoglein 3
- c) Phospholipase A2 receptor
- d) Streptococcal M protein

Q.3) A 48-year-old man with heavy alcohol use presents with severe epigastric pain radiating to the back, vomiting, and elevated lipase. CT shows pancreatic inflammation. Which electrolyte abnormality may occur due to fat necrosis?

- a) Hypercalcemia
- b) Hypocalcemia
- c) Hyponatremia
- d) Hypermagnesemia

Q.4) A hospitalized elderly patient develops watery diarrhea after clindamycin. Colonoscopy shows yellow-white plaques. Stool toxin assay is positive. Most appropriate treatment?

- a) Loperamide
- b) Oral vancomycin
- c) Ceftriaxone
- d) Metronidazole for all severe cases as first choice

Q.5) A 64-year-old chronic smoker is admitted with confusion, headache, and a generalized seizure. He has no edema, no orthostatic hypotension, and no signs of dehydration. Laboratory evaluation shows serum sodium 114 mEq/L, low serum osmolality, urine osmolality of 550 mOsm/kg, and urine sodium of 72 mEq/L. Chest CT shows a central hilar mass. His renal, thyroid, and adrenal functions are normal.

Which mechanism best explains his electrolyte abnormality?

- a) Excess aldosterone secretion causing sodium retention
- b) Ectopic ADH secretion causing water retention
- c) Primary renal tubular sodium wasting with hypovolemia
- d) Osmotic diuresis due to uncontrolled diabetes mellitus

Q.6) A 57-year-old strict vegan presents with progressive numbness in feet, difficulty walking in the dark, memory problems, and fatigue. Examination shows pallor, loss of vibration and position sense, positive Romberg sign,



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spasticity in lower limbs, and hyperreflexia. CBC reveals macrocytic anemia. Methylmalonic acid and homocysteine are elevated. Which neurological structure is primarily affected?

- a) Posterior columns and lateral corticospinal tracts
- b) Spinothalamic tracts only
- c) Anterior horn cells only
- d) Cerebellar Purkinje cells only

Q.7) A 4-year-old boy develops sudden periorbital edema that progresses to generalized swelling. Urine dipstick shows 4+ protein but no blood. Serum albumin is low, cholesterol is elevated, renal function is preserved, and complement levels are normal. He improves dramatically after corticosteroids. Light microscopy is nearly normal. What is the expected electron microscopy finding?

- a) Diffuse podocyte foot process effacement
- b) Subepithelial humps
- c) Linear IgG along glomerular basement membrane
- d) Mesangial IgA deposition

Q.8) A 23-year-old woman has chronic diarrhea, weight loss, low-grade fever, right lower quadrant pain, and recurrent painful perianal drainage. Colonoscopy shows patchy ulcerations separated by normal mucosa. Biopsy reveals transmural inflammation and noncaseating granulomas. Which complication is most characteristic?

- a) Fistula formation
- b) Isolated toxic megacolon as the dominant complication
- c) Continuous mucosal disease beginning at rectum
- d) Pseudopolyps without transmural extension

Q.9) A 34-year-old man who injects heroin presents with fever, pleuritic chest pain, cough, and hemoptysis. Blood cultures grow *Staphylococcus aureus*. Chest CT shows multiple peripheral cavitory nodules consistent with septic pulmonary emboli. A new systolic murmur is heard at the lower left sternal border. Which valve is most likely involved?

- a) Mitral valve
- b) Aortic valve
- c) Tricuspid valve
- d) Pulmonary valve

Q.10) A 46-year-old woman presents with fatigue, weight gain, constipation, cold intolerance, dry skin, and a firm painless enlarged thyroid. TSH is elevated, free T4 is low, and anti-thyroid peroxidase antibodies are positive. Fine-needle aspiration shows oncocytic epithelial cells. Which histologic finding is most characteristic?

- a) Lymphocytic infiltration with Hurthle cells
- b) Orphan Annie eye nuclei and psammoma bodies
- c) Amyloid stroma with calcitonin-positive cells
- d) Sheets of anaplastic pleomorphic giant cells only



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Q.11) A 24-year-old woman presents with photosensitive rash, oral ulcers, arthritis, fever, edema, and dark urine. Labs show anemia, leukopenia, positive ANA, high anti-dsDNA titer, low complement levels, proteinuria, and RBC casts. Kidney biopsy is performed. Which immunofluorescence pattern is most characteristic?

- a) Full-house immunofluorescence
- b) Linear IgG only
- c) Isolated mesangial IgA deposition
- d) Congo red-positive apple-green birefringence

Q.12) A 48-year-old woman has Raynaud phenomenon, dysphagia, tight shiny skin over fingers, telangiectasias, and pulmonary hypertension. She also has calcinosis cutis. ANA is positive. Which antibody is most associated with this pattern?

- a) Anti-centromere antibody
- b) Anti-dsDNA antibody
- c) Anti-histone antibody
- d) Anti-cyclic citrullinated peptide antibody

Q.13) A 43-year-old woman has symmetric swelling and tenderness of MCP and PIP joints, morning stiffness lasting two hours, fatigue, and elevated inflammatory markers. X-ray shows marginal erosions and periarticular osteopenia. Anti-CCP antibodies are positive. Which cytokine is a major therapeutic target?

- a) TNF-alpha
- b) IL-4
- c) Erythropoietin
- d) Interferon-beta only

Q.14) A 68-year-old man has chronic knee pain worse with walking and climbing stairs, relieved by rest. Examination reveals crepitus, bony enlargement, and reduced range of motion without marked warmth. X-ray shows asymmetric joint-space narrowing, osteophytes, subchondral sclerosis, and subchondral cysts.

Most likely diagnosis?

- a) Rheumatoid arthritis
- b) Osteoarthritis
- c) Septic arthritis
- d) Acute gouty arthritis

Q.15) A 58-year-old diabetic male presents with sudden-onset crushing retrosternal chest pain radiating to the left arm and jaw for 90 minutes. He is sweating profusely and appears anxious. ECG shows ST depression in V1–V3 with tall R waves and upright T waves in the same leads. Standard inferior and lateral leads do not show obvious ST elevation. Troponin is initially borderline. The emergency resident considers this as “NSTEMI,” but the cardiologist requests posterior leads V7–V9 immediately.

Which diagnosis is most likely?

- a) NSTEMI due to subendocardial ischemia
- b) Acute posterior wall myocardial infarction
- c) Unstable angina without myocardial necrosis



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d) Acute pericarditis

Q.16) A 38-year-old man with a history of IV drug use presents with fever, weight loss, night sweats, and malaise. Examination shows splinter hemorrhages, Janeway lesions, and a new murmur. Urinalysis shows microscopic hematuria and proteinuria. Blood cultures grow *Staphylococcus aureus*. Echocardiography reveals a vegetation on the tricuspid valve. What best explains the renal abnormality?

- a) Direct bacterial invasion of renal tubules
- b) Immune complex-mediated glomerulonephritis
- c) Acute uric acid nephropathy
- d) Renal artery embolism in all cases

Q.17) A 49-year-old man previously treated for tuberculosis presents with progressive abdominal distension, ankle swelling, fatigue, and exertional dyspnea. Examination shows raised JVP with prominent y descent, hepatomegaly, ascites, peripheral edema, and a pericardial knock. Kussmaul sign is present. Echocardiography shows preserved systolic function but impaired ventricular filling. What is the most likely diagnosis?

- a) Dilated cardiomyopathy
- b) Constrictive pericarditis
- c) Acute pulmonary embolism
- d) Mitral stenosis

Q.18) A 38-year-old woman with childhood history of untreated recurrent sore throat presents with progressive dyspnea, palpitations, and two episodes of hemoptysis. Examination reveals an irregularly irregular pulse, loud first heart sound, opening snap, and low-pitched mid-diastolic rumbling murmur at the apex, best heard in the left lateral position. Chest X-ray shows atrial enlargement. Which complication is she at highest risk for?

- a) Paradoxical embolism from patent foramen ovale only
- b) Left atrial thrombus with systemic embolization
- c) Pulmonary valve stenosis
- d) Aortic dissection

Q.19) A 29-year-old Asian man presents with sudden inability to stand after a carbohydrate-heavy dinner. He had palpitations, weight loss, heat intolerance, and tremor for several months. Examination shows flaccid weakness of proximal lower limbs, preserved sensation, and no bowel or bladder symptoms. Serum potassium is 2.3 mEq/L. TSH is suppressed and free T4 is elevated.

What is the most likely mechanism of weakness?

- a) Autoimmune destruction of anterior horn cells
- b) Intracellular shift of potassium
- c) Loss of potassium through severe diarrhea
- d) Demyelination of peripheral nerves



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Q.20) A 55-year-old man with bipolar disorder treated with lithium presents with polyuria and polydipsia. Serum sodium is 150 mEq/L, serum osmolality is high, and urine osmolality is low. Water deprivation fails to concentrate urine. What is the diagnosis?

- a) Central diabetes insipidus
- b) Nephrogenic diabetes insipidus
- c) SIADH
- d) Psychogenic polydipsia

Q.21) A 56-year-old man with alcoholic cirrhosis and tense ascites presents with fever, worsening abdominal discomfort, confusion, and reduced urine output. Examination shows jaundice, spider angiomas, shifting dullness, and mild diffuse abdominal tenderness without guarding. Diagnostic paracentesis shows ascitic neutrophil count of 420 cells/mm³. What is the most appropriate treatment?

- a) Wait for ascitic culture before starting antibiotics
- b) Immediate IV third-generation cephalosporin
- c) Emergency laparotomy for all cases
- d) Oral lactulose only

Q.22) A 60-year-old man with known cirrhosis is brought with sleep reversal, confusion, asterixis, and fetor hepaticus after an upper GI bleed. He is disoriented but has no focal neurologic deficit. Labs show elevated ammonia. The physician starts treatment to reduce intestinal ammonia production and absorption. Which drug is most appropriate initially?

- a) Lactulose
- b) Acetazolamide
- c) Furosemide
- d) Levodopa

Q.23) A 26-year-old woman presents with recurrent severe abdominal pain, vomiting, constipation, dark urine, anxiety, confusion, and peripheral neuropathy. Attacks are precipitated by fasting and certain medications. Abdomen is soft with minimal tenderness. Urine porphobilinogen is elevated during attacks. What is the most likely diagnosis?

- a) Acute intermittent porphyria
- b) Familial Mediterranean fever
- c) Lead poisoning only
- d) Irritable bowel syndrome

Q.24) A 73-year-old woman presents with new-onset unilateral temporal headache, scalp tenderness while combing hair, jaw pain while chewing, fatigue, and transient visual blurring. ESR is markedly elevated. Examination reveals tenderness and reduced pulsation of the temporal artery. What is the best immediate management?

- a) Wait for biopsy before treatment
- b) Start high-dose glucocorticoids
- c) Give only NSAIDs and review after 2 weeks
- d) Start anticoagulation alone

Q.25) A 70-year-old man develops hypotension after GI bleed. He becomes oliguric. Labs show creatinine rising. Urine sodium is 8 mEq/L, urine osmolality is high, and FeNa is <1%. What is the most likely diagnosis?

- a) Acute tubular necrosis
- b) Prerenal acute kidney injury
- c) Postrenal obstruction
- d) Glomerulonephritis



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Q.26) A 45-year-old man with nephrotic syndrome presents with sudden onset flank pain and hematuria. Doppler shows renal vein thrombosis. What predisposed him?

- a) Increased antithrombin III
- b) Loss of antithrombin III in urine
- c) Decreased fibrinogen
- d) Increased platelet destruction

Q.27) A 76-year-old man presents with exertional chest pain, progressive dyspnea, and two episodes of syncope while climbing stairs. Examination reveals a slow-rising carotid pulse, narrow pulse pressure, and a harsh crescendo-decrescendo systolic murmur at the right second intercostal space radiating to the carotids. ECG shows ventricular hypertrophy. Which condition best explains his symptoms?

- a) Severe aortic stenosis
- b) Mitral regurgitation
- c) Hypertrophic obstructive cardiomyopathy
- d) Pulmonary stenosis

Q.28) A 59-year-old man presents with inferior STEMI. After receiving nitroglycerin, he develops profound hypotension. Examination shows clear lung fields, raised JVP, and bradycardia. ECG right-sided leads show ST elevation in V4R. Which management principle is most appropriate?

- a) Aggressive preload reduction with nitrates
- b) IV fluids
- c) High-dose diuretics immediately
- d) Avoid reperfusion therapy

Q.29) A 68-year-old woman in septic shock initially develops oliguria. After hemodynamic stabilization, urine output improves but creatinine continues to rise. Urinalysis reveals muddy brown granular casts. Urine sodium is elevated, and FeNa is 3.5%. Which diagnosis is most likely?

- a) Prerenal AKI
- b) Acute tubular necrosis
- c) Minimal change disease
- d) SIADH

Q.30) A 28-year-old woman with Sjögren syndrome presents with recurrent kidney stones, muscle weakness, and fatigue. Labs show normal anion gap metabolic acidosis, hypokalemia, and urine pH persistently above 5.5 despite systemic acidosis. Which diagnosis is most likely?

- a) Type 1 distal renal tubular acidosis
- b) Type 2 proximal renal tubular acidosis
- c) Type 4 renal tubular acidosis
- d) Lactic acidosis

Q.31) A 19-year-old woman at 11 weeks by dates presents with severe nausea, vaginal bleeding, and a uterus measuring 17 weeks. β -hCG is markedly elevated. Ultrasound shows a heterogeneous intrauterine mass with multiple cystic spaces. Which genetic mechanism most likely produced this condition?

- a) Fertilization of a normal ovum by one sperm followed by normal embryogenesis
- b) Fertilization of an empty ovum by one sperm that duplicates its chromosomes
- c) Fertilization of a normal ovum by two sperm



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d) Trisomy 21 due to meiotic nondisjunction

Q.32) A 24-year-old primigravida at 33 weeks presents with headache, visual blurring, epigastric pain, and swelling. BP is 168/112 mmHg on repeated measurements. Urine protein is 3+. Platelets are 82,000/mm³, AST/ALT are elevated, and creatinine is increased. Most appropriate diagnosis?

- a) Gestational hypertension
- b) Preeclampsia without severe features
- c) Preeclampsia with severe features
- d) Chronic hypertension only

Q.33) A 30-year-old woman at 32 weeks presents with intense generalized itching, worse on palms and soles, especially at night. There is no rash except excoriations. Liver enzymes are mildly elevated and serum bile acids are increased. Most important fetal risk?

- a) Neural tube defect
- b) Sudden intrauterine fetal demise
- c) Shoulder dystocia
- d) Congenital rubella syndrome

Q.34) A 38-week pregnant woman with poorly controlled gestational diabetes delivers the fetal head after prolonged second stage. The head retracts tightly against the perineum, and gentle traction fails to deliver the shoulders. Best initial maneuver?

- a) Fundal pressure
- b) McRoberts maneuver with suprapubic pressure
- c) Immediate cesarean section after head delivery
- d) Forceful downward traction

Q.35) 36-year-old multiparous woman develops sudden respiratory distress, cyanosis, hypotension, and seizures during labor. Soon after, she develops massive bleeding from IV sites and uterus. Labs show prolonged PT/PTT, low fibrinogen, and thrombocytopenia. Most likely diagnosis?

- a) Pulmonary thromboembolism
- b) Amniotic fluid embolism
- c) Eclampsia alone
- d) Septic shock

Q.36) A 62-year-old man with long-standing hypertension and poorly controlled type 2 diabetes presents with sudden, painless loss of vision in his right eye for 2 hours. He describes it as a curtain falling over the eye. Visual acuity is markedly reduced. Fundus examination shows a pale, edematous retina with a prominent cherry-red spot at the macula. The retinal arteries appear narrowed and attenuated. There is no eye pain, no conjunctival injection, and no photophobia. Carotid bruit is heard on the right side. Which of the following is the most likely diagnosis?

- a) Central retinal artery occlusion
- b) Central retinal vein occlusion
- c) Acute angle-closure glaucoma
- d) Optic neuritis

Q.37) A 28-year-old man presents with acute painful swelling at the eyelid margin. The area is red, tender, and localized. There is no proptosis or painful eye movement. What is the most likely diagnosis?

- a) Hordeolum
- b) Chalazion
- c) Retinal detachment
- d) CRAO



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Q.38) A 24-year-old contact lens wearer presents with severe eye pain, photophobia, tearing, and reduced vision. Slit-lamp examination shows a corneal epithelial defect with stromal infiltrate. There is circumferential injection. Fluorescein staining highlights the ulcer. What is the most likely diagnosis?

- a) Bacterial keratitis
- b) Viral conjunctivitis
- c) Cataract
- d) Retinal detachment

Q.39) A 66-year-old man is found on routine examination to have elevated intraocular pressure in both eyes. He denies pain, redness, halos, nausea, or acute vision loss. Automated perimetry shows progressive peripheral visual field loss. Fundoscopy reveals enlarged cup-to-disc ratio with thinning of the neuroretinal rim. Gonioscopy reveals open anterior chamber angles. Which diagnosis is most likely?

- a) Primary open-angle glaucoma
- b) Acute angle-closure glaucoma
- c) Cataract
- d) Papilledema from intracranial hypertension

Q.40) A 79-year-old woman reports difficulty reading and states that door frames appear wavy. Peripheral vision is preserved. Fundus examination shows macular drusen and pigmentary changes. Fluorescein angiography later demonstrates leakage from choroidal neovascular membranes. Which diagnosis is most likely?

- a) Macular degeneration
- b) Retinal detachment
- c) Optic neuritis
- d) Central retinal vein occlusion

Q.41) A 37-year-old woman presents with a slowly enlarging painless lump in the upper eyelid for 6 weeks. It is firm, non-tender, located within the tarsal plate, and not associated with fever or diffuse eyelid cellulitis. She reports a similar lesion in the past. Which diagnosis is most likely?

- a) Chalazion
- b) Hordeolum
- c) Orbital cellulitis
- d) Dacryocystitis

Q.42) A 72-year-old man develops severe eye pain, marked reduction in vision, eyelid swelling, conjunctival injection, hypopyon, and vitritis 4 days after cataract surgery. The red reflex is poor, and the eye is very tender. Which diagnosis is most likely?

- a) Acute endophthalmitis
- b) Simple subconjunctival hemorrhage
- c) Dry eye syndrome
- d) Posterior vitreous detachment

Q.43) A 76-year-old woman presents with sudden painless monocular vision loss. For several weeks she has had scalp tenderness, jaw claudication, malaise, low-grade fever, and proximal shoulder aching. Fundoscopy shows a pale swollen optic disc. ESR and CRP are markedly elevated. What is the most appropriate immediate management?

- a) Temporal artery biopsy before treatment
- b) High-dose systemic corticosteroids



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- c) Topical antibiotic drops
- d) Observation because vision loss is painless

Q.44) A 64-year-old myopic woman reports sudden onset of floaters and brief flashes in one eye. Visual acuity is normal, and she denies a curtain or field defect. Dilated fundus examination shows a Weiss ring but no retinal tear or detachment. Which diagnosis is most likely?

- a) Posterior vitreous detachment
- b) Rhegmatogenous retinal detachment
- c) Central retinal artery occlusion
- d) Optic neuritis

Q.45) A 45-year-old man reports gradual difficulty seeing objects on both sides while driving. He also has decreased libido and headaches. Visual field testing shows bitemporal hemianopia. MRI reveals a sellar mass extending superiorly. Which structure is compressed?

- a) Optic chiasm
- b) Optic tract
- c) Lateral geniculate body
- d) Occipital cortex

Q.46) A 70-year-old man with atrial fibrillation develops sudden inability to see objects on the right side of space from both eyes. Visual acuity is preserved. Pupils react normally. MRI shows left occipital infarction in the posterior cerebral artery territory. Which visual field defect is expected?

- a) Right homonymous hemianopia
- b) Bitemporal hemianopia
- c) Left monocular blindness
- d) Left superior quadrantanopia only

Q.47) A 39-year-old man develops fever, severe headache, chemosis, proptosis, and painful ophthalmoplegia after a facial skin infection near the upper lip. Examination shows impaired lateral eye movement, decreased corneal reflex, and bilateral eye involvement beginning to develop. Which diagnosis is most likely?

- a) Cavernous sinus thrombosis
- b) Preseptal cellulitis
- c) Chalazion
- d) Central retinal artery occlusion

Q.48) A 69-year-old man receiving intravitreal anti-VEGF injections for wet AMD develops severe pain, profound visual decline, conjunctival injection, hypopyon, and vitritis 3 days after an injection. Which is the most likely diagnosis?

- a) Infectious endophthalmitis
- b) Dry eye disease
- c) Allergic conjunctivitis
- d) Posterior vitreous detachment

Q.49) A 23-year-old man with AIDS presents with blurred vision and floaters. Fundoscopy shows focal necrotizing retinitis adjacent to an old pigmented chorioretinal scar. He has a history of congenital infection. Which diagnosis is most likely?

- a) Toxoplasma chorioretinitis
- b) HSV epithelial keratitis
- c) Cataract
- d) Acute angle-closure glaucoma



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Q.50) A 38-year-old man with advanced HIV and CD4 count of $28/\text{mm}^3$ presents with painless blurred vision and floaters. Fundoscopy shows fluffy white retinal necrosis with hemorrhages along vascular arcades. Which diagnosis is most likely?

- a) CMV retinitis
- b) Retinitis pigmentosa
- c) Retinal detachment
- d) Bacterial conjunctivitis

Q.51) A 38-week infant born by elective cesarean section without labor develops mild retractions 2 hours after birth. Oxygen saturation improves with minimal oxygen. Chest X-ray shows hyperinflation, prominent pulmonary vascular markings, and fluid in the interlobar fissures. Symptoms improve over the next 36 hours. Most likely diagnosis?

- a) Transient tachypnea of newborn
- b) Respiratory distress syndrome
- c) Neonatal pneumonia
- d) Persistent pulmonary hypertension of newborn

Q.52) A 6-year-old boy is evaluated for hypertension. BP is 150/88 mmHg in both arms but 92/60 mmHg in the legs. Femoral pulses are delayed and weak. A systolic murmur is heard over the back. Chest X-ray shows rib notching. Most likely diagnosis?

- a) Coarctation of the aorta
- b) Patent ductus arteriosus
- c) Tetralogy of Fallot
- d) Atrial septal defect

Q.53) A 10-month-old infant with cyanotic congenital heart disease develops sudden deep cyanosis, irritability, and tachypnea while crying. Squatting or knee-chest positioning improves symptoms. Examination shows a harsh systolic murmur at the left upper sternal border. Which defect explains the cyanosis?

- a) Right-to-left shunting due to right ventricular outflow obstruction and VSD
- b) Isolated left-to-right shunt through ASD
- c) PDA causing only continuous murmur without cyanosis
- d) Coarctation causing lower extremity hypertension

Q.54) A 2-year-old child has low-grade fever, barking cough, inspiratory stridor, and hoarse voice. Symptoms worsen at night. He is non-toxic and has no drooling. Neck X-ray shows subglottic narrowing. Most likely diagnosis?

- a) Croup
- b) Epiglottitis
- c) Foreign body aspiration
- d) Bacterial tracheitis

Q.55) A 5-month-old infant in winter presents with rhinorrhea, cough, wheezing, tachypnea, subcostal retractions, and poor feeding. Oxygen saturation is 89%. Chest X-ray shows hyperinflation and peribronchial thickening. Nasal testing detects RSV. Most appropriate management?

- a) Supportive care with oxygen and hydration as needed



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- b) Immediate broad-spectrum antibiotics for all cases
- c) High-dose inhaled corticosteroids as definitive therapy
- d) Antitubercular therapy

Q.56) A 6-year-old boy develops palpable purpura over the buttocks and lower extremities after an upper respiratory infection. He also has colicky abdominal pain, arthralgia, and hematuria. Platelet count and coagulation studies are normal. Which diagnosis is most likely?

- a) IgA vasculitis
- b) Immune thrombocytopenic purpura
- c) Hemophilia A
- d) Acute lymphoblastic leukemia

Q.57) A 7-year-old boy develops fever, sore throat, tender anterior cervical lymph nodes, strawberry tongue, and a diffuse sandpaper-like rash that is accentuated in skin folds. Several days later, desquamation occurs. Rapid antigen test is positive for group A streptococcus. Which toxin mediates the rash?

- a) Erythrogenic exotoxin
- b) Shiga toxin
- c) Exfoliative toxin from
- d) Tetanospasmin from

Q.58) A 3-year-old boy is brought for painless passage of maroon-colored stools. He is pale but afebrile. Abdomen is soft and non-tender. Technetium-99m pertechnetate scan shows uptake in the distal ileum. Which structure failed to involute?

- a) Urachus
- b) Vitelline duct
- c) Allantois
- d) Omphalomesenteric artery only

Q.59) 4-day-old neonate suddenly develops bilious vomiting, abdominal distension, lethargy, and shock. Upper GI contrast study shows a corkscrew appearance of the duodenum and jejunum. Which diagnosis is most likely?

- a) Hypertrophic pyloric stenosis
- b) Midgut volvulus
- c) Gastroesophageal reflux
- d) Meconium ileus

Q.60) A male infant is born at 29 weeks by emergency cesarean section because of severe preeclampsia. Within 30 minutes of birth, he develops tachypnea, nasal flaring, grunting, intercostal retractions, and cyanosis. Chest X-ray shows low lung volumes with diffuse reticulogranular ground-glass opacities and air bronchograms. Arterial blood gas shows hypoxemia and respiratory acidosis. Which is the most likely underlying mechanism?

- a) Delayed absorption of fetal lung fluid
- b) Surfactant deficiency due to immature type II pneumocytes
- c) Meconium aspiration causing airway obstruction
- d) Congenital diaphragmatic hernia causing lung compression



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Q.61) An 18-month-old male presents with fever, drooling, and respiratory distress. In the ED the patient has a temperature of 38.9° C, heart rate of 167 beats/min, blood pressure 95/45 mm Hg, respiratory rate of 35 breaths/min, and oxygen saturation of 96% in room air. You obtain a one-view chest radiograph that shows a circular foreign body lodged in the patient's esophagus. The next best step in evaluating this patient is to:

- a) Make the patient NPO and obtain a two-view chest radiograph
- b) Place a peripheral intravenous line and give ceftriaxone
- c) Consult specialist for urgent endoscopy
- d) Admit for monitoring of potential complications

Q.62) A 3-week-old neonate is brought to the pediatric clinic due to poor feeding and excessive sleepiness. The infant was born at term via uncomplicated vaginal delivery. On physical examination, the clinician notes a large posterior fontanelle, a protruding tongue, and a reducible umbilical hernia. The infant appears mildly icteric. A screening test performed at 48 hours of life showed a capillary TSH of 80 mU/L. Which of the following is the most likely long-term consequence if this condition remains untreated?

- a) Precocious puberty and accelerated bone age
- b) Irreversible sensorineural hearing loss and intellectual disability
- c) Hypertrophic cardiomyopathy with left ventricular outflow tract obstruction
- d) Increased risk of papillary thyroid carcinoma in early childhood

Q.63) A 6-year-old boy is brought by his parents because of a painless limp for 2 months. He occasionally complains of vague pain in the groin and medial thigh, and the pain sometimes seems to radiate to the knee. There is no history of trauma, fever, weight loss, or recent infection. Examination shows an antalgic gait, limited hip abduction, and markedly reduced internal rotation of the right hip. Laboratory tests, including WBC count, ESR, and CRP, are normal. Pelvic radiograph shows flattening, sclerosis, and fragmentation of the right femoral head epiphysis.

Which of the following is the most likely diagnosis?

- a) Slipped capital femoral epiphysis
- b) Septic arthritis of the hip
- c) Legg-Calvé-Perthes disease
- d) Developmental dysplasia of the hip

Q.64) A 13-year-old boy who plays competitive soccer presents with progressively worsening anterior knee pain over the past 3 months. The pain is localized just below the patella and is exacerbated by running, jumping, and climbing stairs. He reports no history of acute trauma. Physical examination reveals a prominent, tender swelling over the tibial tuberosity. Pain is reproduced with resisted knee extension and direct palpation of the tibial tubercle. There is full range of motion of the knee, and no joint effusion is noted. X-ray of the knee shows fragmentation and irregularity of the tibial tuberosity apophysis.



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Which of the following is the most likely diagnosis?

- a) Osgood–Schlatter disease
- b) Patellar tendon rupture
- c) Tibial tubercle fracture
- d) Septic arthritis of the knee

Q.65) A 3-year-old boy has fever for 6 days, bilateral nonpurulent conjunctivitis, cracked red lips, strawberry tongue, polymorphous rash, swollen hands and feet, and unilateral cervical lymphadenopathy. Labs show elevated ESR/CRP and thrombocytosis. Echocardiography is ordered. Most important complication?

- a) Coronary artery aneurysm
- b) Rheumatic mitral stenosis
- c) Pulmonary fibrosis
- d) Renal agenesis

Q.66) A 13-year-old obese boy presents with a 2-month history of dull left hip pain and intermittent limp. He denies trauma. Pain worsens with activity and occasionally radiates to the knee. Examination reveals:



- External rotation of the affected leg
- Limited internal rotation
- Antalgic gait

X-ray (frog-leg lateral) shows displacement of the femoral head epiphysis posteriorly and inferiorly relative to the femoral neck. Klein's line fails to intersect the femoral head.

What is the most likely diagnosis?

- a) Legg–Calvé–Perthes disease
- b) Slipped capital femoral epiphysis
- c) Septic arthritis
- d) Transient synovitis

Q.67) A 17-year-old boy presents with progressive pain and swelling around the distal femur. Pain worsens at night and is not relieved by rest. Examination reveals a firm tender mass.



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X-ray shows:

- Mixed lytic and sclerotic lesion
- periosteal reaction
- Codman triangle

What is the most likely diagnosis?

- a) Ewing sarcoma
- b) Osteosarcoma
- c) Osteomyelitis
- d) Bone metastasis

Q.68) A 68-year-old woman falls on an outstretched hand. She complains of wrist pain and deformity.

X-ray shows:

- Distal radius fracture
- Dorsal displacement and angulation
- What is the diagnosis?

- a) Smith fracture
- b) Colles fracture
- c) Scaphoid fracture
- d) Monteggia fracture

Q.69) A 31-year-old man is in a dashboard injury during a car crash. His right hip is flexed, adducted, and internally rotated. X-ray shows posterior dislocation of the femoral head. He has weakness of ankle dorsiflexion and numbness over the dorsum of the foot. Which nerve is most likely injured?

- a) Femoral nerve
- b) Sciatic nerve
- c) Obturator nerve
- d) Superior gluteal nerve



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Q.70) A 24-year-old man falls on an outstretched hand. He has radial-sided wrist pain and marked tenderness in the anatomic snuffbox. Initial wrist X-ray is normal. The physician is concerned about avascular necrosis if the fracture is missed. Most appropriate management?

- a) Reassure and allow full activity
- b) Immobilize in thumb spica
- c) Immediate amputation
- d) Treat as carpal tunnel syndrome

Q.71) A 12-year-old boy has fever, elevated ESR, leukocytosis, and severe pain in the midshaft femur. X-ray shows a permeative diaphyseal lesion with periosteal reaction. Biopsy shows small round blue cells with t(11;22).

Diagnosis?

- | | |
|-----------------------|---------------------|
| a) Osteomyelitis only | c) Chondrosarcoma |
| b) Ewing sarcoma | d) Giant cell tumor |

Q.72) A 78-year-old man presents with severe itching followed by multiple tense bullae over the abdomen, groin, and flexural surfaces. The blisters remain intact for several days and do not rupture easily. Oral mucosa is normal. Nikolsky sign is negative. Skin biopsy shows a subepidermal blister with eosinophils. Direct immunofluorescence shows linear IgG and C3 along the basement membrane.

Which antigenic target is most likely involved?

- a) Desmoglein 3
- b) Hemidesmosomal proteins BP180 and BP230
- c) Epidermal transglutaminase
- d) Type VII collagen

Q.73) A 34-year-old man presents with chronic plaques over extensor surfaces. Scraping reveals pinpoint bleeding. Biopsy shows parakeratosis, acanthosis, elongation of rete ridges, and neutrophils in stratum corneum. Which cytokine is most implicated?

- | | |
|----------|-----------------|
| a) IL-17 | c) IL-10 |
| b) IL-4 | d) TGF- β |

Q.74) A 7-year-old boy with asthma and allergic rhinitis has recurrent intensely pruritic eczematous plaques in the antecubital and popliteal fossae. His mother reports that scratching worsens the rash and causes thickened skin. Examination shows xerosis, lichenification, and excoriations. Serum IgE is elevated.

Which mechanism best explains this disease?

- a) Defective skin barrier with Th2-predominant inflammation
- b) Autoantibodies against hemidesmosomes



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- c) Dermatophyte invasion of stratum corneum
- d) Malignant proliferation of melanocytes

Q.75) A 63-year-old man presents with a changing pigmented lesion on the upper back. It is 11 mm, asymmetric, has irregular borders, multiple shades of brown and black, and has recently bled. Dermoscopy shows atypical pigment network. Excisional biopsy confirms malignant melanoma.

Which feature is the most important prognostic factor?

- a) Breslow thickness
- b) Surface color only
- c) Degree of itching
- d) Presence of hair within lesion

Q.76) A 26-year-old medical student reports a 9-day period during which he slept 2 hours per night, felt unusually energetic, spent ₹2 lakh on unnecessary equipment, spoke rapidly, believed he had developed a “revolutionary cure,” and was hospitalized after attempting to enter a restricted laboratory. He later develops a 3-week episode of low mood, anhedonia, guilt, hypersomnia, and suicidal ideation.

Which diagnosis is most likely?

- a) Major depressive disorder
- b) Bipolar I disorder
- c) Bipolar II disorder
- d) Cyclothymic disorder

Q.77) A 45-year-old woman presents with 2 months of profound depression. She has lost interest in all activities, including those she previously enjoyed. She wakes at 4 AM daily, has severe morning worsening of mood, psychomotor retardation, excessive guilt, and 7-kg weight loss. She denies hallucinations or manic episodes.

Which specifier best describes this episode?

- a) Atypical features
- b) Melancholic features
- c) Catatonic schizophrenia
- d) Persistent depressive disorder

Q.78) A 17-year-old girl has lost 18 kg in 8 months. BMI is 15.2 kg/m². She fears gaining weight despite being underweight and sees herself as “fat.” She restricts food, runs excessively, and has amenorrhea. Labs show hypokalemia and low estrogen. She denies recurrent binge episodes. Which diagnosis is most likely?

- a) Bulimia nervosa
- b) Anorexia nervosa
- c) Binge-eating disorder
- d) Body dysmorphic disorder

Q.79) A 44-year-old man has chronic abdominal pain, fatigue, headaches, and back pain for several years. Extensive evaluation is unrevealing. He spends many hours daily checking symptoms, repeatedly seeks reassurance, and believes doctors are missing a serious illness. His distress is mainly focused on the symptoms themselves.

Which diagnosis is most likely?



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- a) Somatic symptom disorder
- b) Illness anxiety disorder
- c) Factitious disorder
- d) Malingering

Q.80) An 82-year-old hospitalized man becomes acutely confused 2 days after surgery. He fluctuates between agitation and somnolence, cannot sustain attention, misidentifies staff, and has visual hallucinations. His family says he was independent before admission. He is taking opioids and anticholinergic medication.

Which diagnosis is most likely?

- a) Alzheimer disease
- b) Delirium
- c) Schizophrenia
- d) Major depressive disorder with pseudodementia

Q.81) 42-year-old woman receives an interscalene block with bupivacaine. Minutes later, she develops circumoral numbness, metallic taste, tinnitus, agitation, generalized seizure, followed by wide-complex ventricular arrhythmia and hypotension. Best treatment?

- a) IV lipid emulsion
- b) IV protamine sulfate
- c) IV vitamin K
- d) IV atropine only

Q.82) A 50-year-old obese man with obstructive sleep apnea, limited mouth opening, short neck, and restricted neck extension requires elective surgery. Airway assessment suggests difficult mask ventilation and difficult intubation. He is stable and cooperative. Best airway plan?

- a) Induce general anesthesia and paralyze immediately before confirming ventilation
- b) Awake fiberoptic intubation with topical anesthesia and sedation
- c) Avoid all airway planning
- d) Deep sedation without airway equipment available

Q.83) A patient receives rocuronium for laparoscopic surgery. At the end of surgery, rapid reversal is needed. Sugammadex is administered. What is its mechanism?

- a) Acetylcholinesterase inhibition
- b) Encapsulation of aminosteroid neuromuscular blockers
- c) Competitive opioid receptor antagonism
- d) Calcium release inhibition from sarcoplasmic reticulum

Q.84) A 49-year-old woman with myasthenia gravis undergoes thymectomy. The anesthesiologist plans neuromuscular blockade.

Which statement is most accurate?



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- a) She is resistant to nondepolarizing blockers and sensitive to succinylcholine
- b) She is sensitive to nondepolarizing blockers and relatively resistant to succinylcholine
- c) She requires no monitoring because MG does not affect anesthesia
- d) All neuromuscular blockers are absolutely contraindicated

Q.85) A 58-year-old man receives multiple IV morphine doses after abdominal surgery. He becomes somnolent with respiratory rate 6/min, oxygen saturation 84%, pinpoint pupils, and elevated CO₂. Most appropriate reversal agent?

- a) Naloxone
- b) Flumazenil
- c) Neostigmine
- d) Dantrolene

Q.86) A woman treated for complete molar pregnancy returns 3 months later with hemoptysis and persistent vaginal bleeding. β -hCG remains very high despite uterine evacuation. Chest imaging shows multiple pulmonary nodules.

What is the most likely diagnosis?

- a) Endometrial carcinoma
- b) Choriocarcinoma
- c) Placental site trophoblastic tumor
- d) Invasive cervical carcinoma

Q.87) A 32-week pregnant woman has AFI of 4 cm. What does this indicate?

- a) Normal liquor
- b) Oligohydramnios
- c) Polyhydramnios
- d) Borderline high AFI

Q.88) A primigravida has the following cervical findings:

- Dilatation: 3 cm
- Effacement: 60%
- Station: -1
- Consistency: soft
- Position: anterior

What is the Bishop score?

- a) 5
- b) 7
- c) 9
- d) 11

Q.89) A district reports:



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- Maternal deaths = 18
- Live births = 12,000

What is the maternal mortality ratio?

- a) 15 per 100,000 live births
- b) 100 per 100,000 live births
- c) 150 per 100,000 live births
- d) 180 per 100,000 live births

Q.90) A woman develops bleeding immediately after rupture of membranes, followed by sudden fetal bradycardia.

All are correct regarding EXCEPT:

- a) Bleeding is fetal in origin
- b) It is associated with velamentous cord insertion
- c) Maternal shock is usually severe despite minimal fetal distress
- d) Rupture of membranes can precipitate fetal exsanguination

Q.91) All are correct regarding adenomyosis EXCEPT:

- a) Endometrial glands are present within myometrium
- b) Uterus may be uniformly enlarged and boggy
- c) It is classically associated with severe dyspareunia, infertility, and powder-burn peritoneal lesions
- d) It commonly affects multiparous women

Q.92) All are correct regarding uteroplacental circulation EXCEPT:

- a) Uterine blood flow at term is approximately 500–750 mL/min
- b) Spiral arteries lose muscular layer and become low-resistance vessels
- c) Placental circulation is autoregulated like cerebral circulation
- d) Uteroplacental blood flow is dependent on maternal blood pressure

Q.93) During early pregnancy, three decidual layers form as a result of endometrial transformation.

All are correctly matched EXCEPT:

- a) Decidua basalis – implantation site
- b) Decidua capsularis – surrounds embryo
- c) Decidua parietalis – rest of uterine lining
- d) Decidua parietalis – forms fetal membranes



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Q.94) A 36-year-old G5P4 woman with placenta previa and three previous cesarean deliveries undergoes cesarean section. The placenta does not separate after delivery, and attempted removal causes torrential hemorrhage. There is no normal cleavage plane.

Everything is correct EXCEPT:

- a) It is associated with defective decidua basalis
- b) Prior cesarean with placenta previa markedly increases risk
- c) Forceful manual removal is the safest recommended management
- d) Cesarean hysterectomy may be required in severe cases

Q.95) A 21-year-old woman at 11 weeks presents with vaginal bleeding, severe vomiting, uterine size greater than dates, early hypertension, and very high beta-hCG. Ultrasound shows a snowstorm appearance .

Everything is correct regarding this condition EXCEPT:

- a) It is usually diploid and androgenetic
- b) Fetal tissue is typically absent
- c) It has higher malignant potential than partial mole
- d) It is usually triploid

Q.96) Placental morphological variants can predispose to significant obstetric complications depending on their structural configuration and completeness of expulsion. In the case of a Placenta succenturiata, which of the following is least likely to be encountered as a clinical consequence?

- a) Preterm delivery
- b) Postpartum hemorrhage
- c) Retained (missing) placental lobe
- d) Puerperal sepsis and uterine subinvolution

Q.97) During meticulous postpartum examination of the placenta, a well-circumscribed, translucent defect is identified within the placental disc, giving the appearance of a “window” where placental tissue is absent but the surrounding membranes remain intact. This rare morphological variant may be mistaken for a missing cotyledon and has implications for clinical interpretation.

Which of the following best describes this placental abnormality?

- a) Fenestrated placenta
- b) Succenturiate placenta
- c) Circumvallate placenta
- d) Placenta membranacea

Q.98) A 26-year-old G1P0 woman at 39 weeks presents in spontaneous labor. Her membranes rupture clear fluid at 5 cm cervical dilatation. Four hours later, she is 9 cm dilated, 100% effaced, and the fetal head is at 0 station.



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Two more hours later, she remains 9 cm dilated despite adequate uterine contractions documented by intrauterine pressure catheter. Fetal heart tracing is category II with recurrent late decelerations that improve transiently with maternal repositioning and IV fluids. After another hour, there is no cervical change, caput increases, and molding becomes prominent. The fetal head remains at 0 station.

Which of the following is the most appropriate management?

- a) Continue observation because she has not yet entered the second stage of labor
- b) Perform operative vaginal delivery using forceps
- c) Perform cesarean delivery for active-phase arrest with nonreassuring fetal status
- d) Begin maternal pushing because the cervix is almost completely dilated

Q.99) A 29-year-old G2P1 woman at 40 + 2 weeks gestation presents in spontaneous labor. Her previous delivery was vaginal without complications. On admission, cervical examination shows 4 cm dilatation, 80% effacement, and -2 station. Over the next 3 hours, her cervix dilates to 6 cm. An amniotomy is performed, and contractions become regular and adequate (measured at >200 Montevideo units).

Six hours later, the cervix is 8 cm dilated, 100% effaced, and the fetal head is at -1 station. Over the next 4 hours, despite continued adequate contractions, there is no further cervical change, and the fetal head remains at -1 station. Fetal heart tracing is category I. The patient is comfortable with epidural analgesia.

Which of the following best describes her current labor abnormality?

- a) Prolonged latent phase
- b) Arrest of active phase of first stage of labor
- c) Prolonged second stage of labor
- d) Failure of descent

Q.100) A 31-year-old G1P0 woman at 39 weeks gestation is admitted in spontaneous labor. She progresses normally through the first stage and achieves full cervical dilatation (10 cm). An epidural is in place. The fetal head is in occiput anterior position at +1 station. She begins pushing. After 2 hours of pushing, the fetal head is now at +2 station. Fetal heart tracing remains category I. She is not exhausted, and contractions remain adequate.

After one additional hour of pushing, the head remains at +2 station with no further descent. Mild caput and molding are noted. The pelvis appears clinically adequate, and the fetal position remains occiput anterior.

Which of the following is the most appropriate next step in management?

- a) Continue expectant management because duration is still within normal limits
- b) Diagnose arrest of descent and proceed directly to cesarean section
- c) Attempt operative vaginal delivery using vacuum or forceps
- d) Stop pushing and convert to induction of labor



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Q.101) A 35-year-old G3P2 woman at 39 weeks undergoes an uncomplicated vaginal delivery after spontaneous labor. She has a history of one prior cesarean section for fetal distress and one prior normal vaginal delivery. The third stage of labor is actively managed with oxytocin infusion.

Approximately 7 minutes after delivery of the fetus, the following are noted:

- Sudden gush of blood
- Lengthening of the umbilical cord
- Uterine fundus becomes globular and rises above the umbilicus

Controlled cord traction is applied, and a portion of the placenta is delivered. However, the placenta appears irregular and incomplete, with torn membranes and suspicion of a missing accessory lobe.

Within the next 5–10 minutes:

- Vaginal bleeding increases progressively
- Uterus is firm, well contracted, and central
- No obvious vaginal or cervical lacerations are identified
- Patient becomes tachycardic but maintains blood pressure

The obstetrician suspects a structural abnormality of the placenta that predisposed to incomplete expulsion.

Which of the following is the most likely underlying pathology and most appropriate immediate management?

- a) Placenta accreta spectrum; proceed to emergency hysterectomy
- b) Placenta succenturiata; perform manual uterine exploration and removal
- c) Uterine atony; increase oxytocin and perform bimanual massage
- d) Marginal placental separation; continue controlled cord traction

Q.102) A 30-year-old G2P1 woman is referred at 30 weeks gestation for evaluation of marked polyhydramnios and a fetus measuring >95th percentile for gestational age. Detailed ultrasound reveals macroglossia, omphalocele, enlarged kidneys, and placentomegaly. There is no evidence of structural cardiac defect. The mother has no history of diabetes. Amniotic fluid index is 28 cm.

At birth, the neonate develops early severe hypoglycemia requiring IV glucose infusion. On day 3, examination shows asymmetric limb overgrowth and a small ear crease/pit. Genetic testing is sent. Which of the following is the most likely underlying mechanism AND the most important long-term complication to monitor?

- a) Trisomy 21 due to nondisjunction; congenital heart disease
- b) Loss of imprinting at chromosome 11p15; increased risk of embryonal tumors
- c) Turner syndrome mosaicism; gonadal dysgenesis
- d) Fetal alcohol syndrome; neurodevelopmental delay



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Q.103) A woman in labor has vaginal exam showing fetal mouth, nose, and orbital ridges. The mentum is anterior. Best delivery route if labor progresses?

- a) Vaginal delivery possible
- b) Cesarean always mandatory
- c) Vacuum delivery mandatory
- d) Internal podalic version

Q.104) A 28-year-old at 36 weeks has vomiting, jaundice, confusion, abdominal pain, hypoglycemia, coagulopathy, elevated ammonia, leukocytosis, and modest transaminase elevation.

Most likely diagnosis?

- a) HELLP syndrome
- b) Acute fatty liver of pregnancy
- c) Hyperemesis gravidarum
- d) Intrahepatic cholestasis

Q.105) During antenatal imaging and subsequent postpartum examination, the placenta is noted to form a diffuse, thin membranous sheet covering a large proportion of the uterine cavity rather than a well-circumscribed discoid mass. This abnormality is associated with defective localization of chorionic villi and has important clinical implications.

Which of the following best describes this placental condition?

- a) Circumvallate placenta
- b) Placenta succenturiate
- c) Placenta membranacea
- d) Battledore placenta

Q.106) A 27-year-old G1P0 at 40 weeks reaches 6 cm dilation with ruptured membranes. Intrauterine pressure catheter shows adequate contractions >200 Montevideo units. After 4 hours, she remains 6 cm dilated with no cervical change. Fetal tracing is category I. What is the diagnosis?

- a) Prolonged latent phase
- b) Active phase arrest
- c) Prolonged second stage
- d) Arrest of descent

Q.107) A woman at 37 weeks has breech presentation and complete placenta previa. Best management regarding ECV?

- a) Perform ECV immediately
- b) ECV contraindicated
- c) ECV after amniotomy
- d) ECV with fundal pressure during labor



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Q.108) A 22-year-old primigravida at 22 weeks has incidental cervical length 18 mm on transvaginal ultrasound. She has no contractions and no prior preterm birth. Best management?

- a) Vaginal progesterone
- b) Immediate classical caesarean
- c) Tocolysis for 10 weeks
- d) Methotrexate

Q.109) A 17-year-old phenotypic female has primary amenorrhea, normal breast development, scant pubic hair, normal ovaries, and absent uterus/upper vagina. Karyotype is 46,XX. Diagnosis?

- a) Turner syndrome
- b) Müllerian agenesis
- c) Androgen insensitivity syndrome
- d) Imperforate hymen

Q.110) A pregnant woman is 14 weeks pregnant by LMP. What is her approximate conceptional age?

- a) 10 weeks
- b) 12 weeks
- c) 14 weeks
- d) 16 weeks

Q.111) A 52-year-old man sustains blunt chest trauma. He has severe pain and paradoxical inward movement of a chest wall segment during inspiration. Chest X-ray shows multiple adjacent rib fractures in two places.

Primary cause of respiratory failure?

- a) Rib fracture pain only
- b) Underlying pulmonary contusion
- c) Diaphragmatic paralysis
- d) Acute asthma

Q.112) A 48-year-old man develops severe chest pain, fever, and dyspnea after forceful vomiting. Exam shows subcutaneous emphysema in the neck. Chest X-ray shows mediastinal air and left pleural effusion.

Most likely diagnosis?

- a) Mallory-Weiss tear
- b) Boerhaave syndrome
- c) Acute pancreatitis
- d) Perforated peptic ulcer

Q.113) A 22-year-old man has periumbilical pain migrating to the right lower quadrant, anorexia, nausea, low-grade fever, and leukocytosis. Pain worsens with passive extension of the right hip.

Which structure is irritated?

- a) Obturator internus
- b) Psoas muscle
- c) Diaphragm
- d) Rectus abdominis only



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Q.114) A 58-year-old woman presents with acute abdominal distension and pain. CT shows a massively dilated cecum displaced to the left upper quadrant with whirl sign. Endoscopic decompression is unlikely to succeed.

Best management?

- a) Colonoscopic decompression as definitive therapy
- b) Surgical resection
- c) Oral antibiotics only
- d) Nasogastric tube alone

Q.115) A 44-year-old obese woman has persistent right upper quadrant pain, fever, leukocytosis, and positive Murphy sign. Ultrasound shows gallstones, gallbladder wall thickening, and pericholecystic fluid.

Best management?

- a) Elective surgery after 6 months only
- b) Early laparoscopic cholecystectomy with antibiotics
- c) ERCP first in all cases
- d) Oral antacids

Q.116) A 62-year-old woman with gallstones presents with fever, jaundice, RUQ pain, hypotension, and confusion. Labs show elevated bilirubin and alkaline phosphatase.

Best next step?

- a) Elective outpatient cholecystectomy
- b) Broad-spectrum antibiotics and urgent ERCP decompression
- c) Observation only
- d) HIDA scan before treatment

Q.117) A 69-year-old smoker presents with painless jaundice, dark urine, pale stools, pruritus, weight loss, and a palpable non-tender gallbladder.

Most likely diagnosis?

- a) Acute cholecystitis
- b) Pancreatic head adenocarcinoma
- c) Hepatitis A
- d) Peptic ulcer disease

Q.118) A 58-year-old diabetic man presents with fever, perineal pain, scrotal swelling, skin discoloration, and crepitus. He is hypotensive.

Best treatment?



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- a) Topical antifungal cream
- b) Urgent surgical debridement and broad-spectrum antibiotics
- c) Elective hernia repair
- d) Observation

Q.119) A 31-year-old man is brought after a high-speed motor vehicle collision. He was restrained with a lap belt. Initially, he is hemodynamically stable and complains only of vague abdominal discomfort. Examination reveals a horizontal ecchymotic mark across the lower abdomen. FAST is negative. CT abdomen shows small amounts of free intraperitoneal fluid without solid organ injury, mild bowel wall thickening, and mesenteric stranding. Six hours later, he develops increasing abdominal pain, fever, tachycardia, guarding, and leukocytosis.

Which is the most likely diagnosis?

- a) Isolated splenic laceration
- b) Small bowel or mesenteric injury
- c) Acute pancreatitis from alcohol
- d) Simple abdominal wall contusion

Q.120) A 40-year-old man sustains blunt thoracoabdominal trauma. He later develops respiratory distress. Chest X-ray shows elevation of the left hemidiaphragm, bowel gas pattern in the left hemithorax, and mediastinal shift to the right. Breath sounds are reduced on the left.

Most likely diagnosis?

- a) Left diaphragmatic rupture
- b) Tension pneumothorax
- c) Simple rib fracture
- d) Pulmonary contusion only

Q.121) A man with pelvic fracture has gross hematuria and inability to void. Retrograde cystogram shows contrast extravasation around the bladder in a flame-shaped pattern, confined to the pelvis.

Most likely diagnosis?

- a) Intraperitoneal bladder rupture
- b) Extraperitoneal bladder rupture
- c) Ureteric transection
- d) Renal pelvis rupture

Q.122) A 35-year-old intoxicated man is struck in the lower abdomen after drinking heavily. He has lower abdominal pain, inability to void, and gross hematuria. CT cystogram shows contrast outlining loops of bowel. Best management?

- a) Foley catheter drainage alone
- b) Surgical repair
- c) Oral antibiotics only
- d) Immediate nephrectomy



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Q.123) A farm worker sustains a contaminated crush injury to the thigh. Twenty-four hours later he develops severe pain, swelling, bronze discoloration, foul discharge, crepitus, hemolysis, renal failure, and shock. Most likely organism?

- a) Clostridium perfringens
- b) Staphylococcus epidermidis
- c) Candida albicans
- d) Mycobacterium tuberculosis

Q.124) A patient has profuse watery diarrhea, hypokalemia, dehydration, and achlorhydria. Stool persists despite fasting. Diagnosis?

- a) VIPoma
- b) Insulinoma
- c) Gastrinoma
- d) Carcinoid appendix

Q.125) A 57-year-old man has diabetes, weight loss, anemia, glossitis, and a migratory erythematous blistering rash around groin and perineum. Diagnosis?

- a) Insulinoma
- b) Glucagonoma
- c) Gastrinoma
- d) Somatostatinoma

Q.126) A patient with MEN2 has thyroid mass. Histology shows nests of malignant cells with amyloid stroma. Serum calcitonin is elevated.

Diagnosis?

- a) Papillary carcinoma
- b) Follicular carcinoma
- c) Medullary thyroid carcinoma
- d) Hashimoto thyroiditis

Q.127) A young adult has a painless lateral neck mass anterior to the sternocleidomastoid that enlarges after upper respiratory infection. Diagnosis?

- a) Thyroglossal duct cyst
- b) Branchial cleft cyst
- c) Dermoid cyst
- d) Parathyroid adenoma

Q.128) A 62-year-old man presents with altered bowel habits, weight loss, and occult blood-positive stool. Colonoscopy shows an ulceroproliferative mass in the sigmoid colon. Biopsy confirms adenocarcinoma. CT abdomen shows no liver metastasis. He undergoes colectomy. Histopathology shows tumor invading through the muscularis propria into pericolic fat. Three regional lymph nodes are positive. No distant metastasis is found.

According to TNM staging, which stage best describes this cancer?

- a) Stage I
- b) Stage II
- c) Stage III
- d) Stage IV



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Q.129) A patient has colon cancer invading muscularis propria (T2), with 2 positive lymph nodes, no metastasis. Stage?

- a) Stage I
- b) Stage II
- c) Stage III
- d) Stage IV

Q.130) A 44-year-old woman has a screening mammogram showing a suspicious 1.4-cm lesion in the upper outer quadrant of the breast. Core biopsy shows invasive ductal carcinoma. Sentinel lymph node biopsy is negative. CT and bone scan show no metastasis.

What is the most likely anatomic stage grouping?

- a) Stage I
- b) Stage II
- c) Stage III
- d) Stage IV

Q.131) A 67-year-old uncircumcised man is catheterized for urinary retention. Six hours later, he complains of severe penile pain. Examination shows the foreskin retracted behind the glans and unable to return to its normal position. The glans is swollen, congested, and tender. A tight constricting band is seen behind the coronal sulcus.

What is the most likely diagnosis?

- a) Phimosis
- b) Paraphimosis
- c) Balanitis xerotica obliterans
- d) Priapism

Q.132) A 44-year-old woman suddenly develops the worst headache of her life while exercising. She vomits, becomes photophobic, and has neck stiffness. CT head shows blood in the basal cisterns. She is alert but drowsy. CT angiography shows a ruptured anterior communicating artery aneurysm.

Which complication is most classically prevented by nimodipine?

- a) Rebleeding from aneurysm rupture
- b) Cerebral vasospasm causing delayed ischemic deficit
- c) Hydrocephalus due to aqueduct stenosis
- d) Seizure due to temporal lobe scarring

Q.133) A 27-year-old man develops worsening consciousness after traumatic intracranial hemorrhage. Examination shows a dilated right pupil, right eye deviated down and out, left-sided weakness, and progressive bradycardia with hypertension. CT shows right temporal mass effect with medial displacement of the temporal lobe.

Which structure is being compressed first to cause the pupillary abnormality?

- a) Abducens nerve at cavernous sinus
- b) Ipsilateral oculomotor nerve
- c) Contralateral optic tract
- d) Facial nerve in internal auditory canal



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Q.134) A 30-year-old man with severe traumatic brain injury has declining GCS, anisocoria, and CT evidence of cerebral edema. He is intubated. Blood pressure is maintained. The team initiates head elevation, avoids hypoxia and hypotension, and considers osmotherapy.

Which intervention is appropriate for acute intracranial pressure reduction?

- a) Hypotonic IV fluids
- b) Mannitol
- c) Lumbar puncture
- d) Aggressive hypotension

Q.135) A man falls from height and develops periorbital ecchymosis, retroauricular bruising, CSF rhinorrhea, and hemotympanum. Which diagnosis is most likely?

- a) Basilar skull fracture
- b) Isolated nasal bone fracture
- c) Orbital cellulitis
- d) Subgaleal hematoma

Q.136) A 46-year-old man with an undiagnosed pituitary macroadenoma develops sudden severe headache, vomiting, visual loss, ophthalmoplegia, hypotension, and altered consciousness. MRI shows hemorrhage into the pituitary gland. What is the most urgent medical therapy?

- a) High-dose corticosteroids
- b) Oral levothyroxine alone
- c) Dopamine antagonist
- d) Aspirin

Q.137) 63-year-old man presents with new-onset seizures, personality change, headache, and progressive left-sided weakness. MRI shows an irregular ring-enhancing intra-axial mass crossing the corpus callosum with central necrosis. Most likely diagnosis?

- a) Low-grade astrocytoma
- b) Glioblastoma multiforme
- c) Meningioma
- d) Pituitary adenoma

Q.138) A 6-year-old boy presents with morning vomiting, headache, truncal ataxia, and papilledema. MRI shows a midline cerebellar vermis tumor compressing the fourth ventricle, causing obstructive hydrocephalus.

Most likely tumor?

- a) Medulloblastoma
- b) Meningioma
- c) Vestibular schwannoma
- d) Oligodendroglioma

Q.139) A 45-year-old man with severe lumbar disc herniation develops bilateral sciatica, saddle anesthesia, urinary retention, decreased anal tone, and weakness of ankle plantarflexion. MRI shows a large central L4-L5 disc compressing the cauda equina.

Best management?



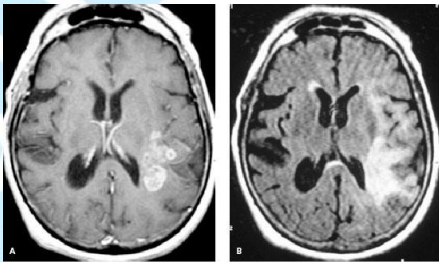
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- a) Oral analgesics and outpatient follow-up
- b) Emergency surgical decompression
- c) Bed rest for 6 weeks
- d) Lumbar puncture

Q.140) A 51-year-old woman presents with sudden severe headache and double vision. Examination shows left ptosis, a dilated nonreactive left pupil, and the left eye is deviated down and out. CT angiography reveals an aneurysm near the junction of the internal carotid and posterior communicating artery. Which nerve is compressed?

- a) Trochlear nerve
- b) Abducens nerve
- c) Oculomotor nerve
- d) Optic nerve

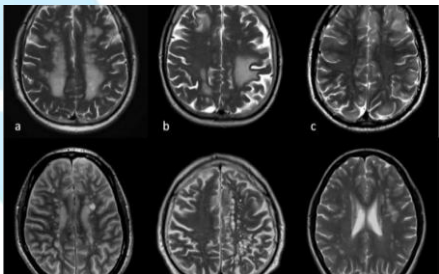


Q.141)

Imaging shows irregular ring-enhancing lesion with central necrosis crossing midline.

Diagnosis?

- a) Abscess
- b) Glioblastoma
- c) Meningioma
- d) Infarct



Q.142)

RI shows periventricular white matter plaques ("Dawson fingers"). Diagnosis?

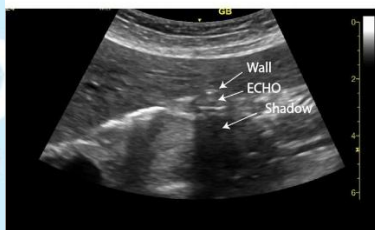
- a) Stroke
- b) Multiple sclerosis
- c) Tumor
- d) Abscess



Q.143)

X-ray shows multiple air-fluid levels and dilated central loops. Diagnosis?

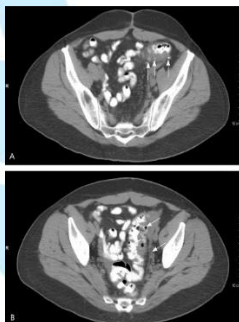
- | | |
|----------------------------|--------------|
| a) Large bowel obstruction | c) Ileus |
| b) Small bowel obstruction | d) Gastritis |



Q.144)

Diagnosis?

- | | |
|-------------------|-----------------|
| a) Cholelithiasis | c) Cirrhosis |
| b) Hepatitis | d) Pancreatitis |



Q.145)

CT shows:

- Sigmoid wall thickening
- Fat stranding

Diagnosis?

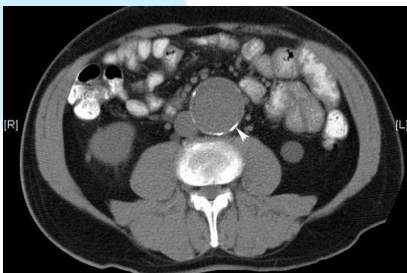
- | | |
|-------------------|-----------|
| a) Diverticulitis | c) Cancer |
| b) Appendicitis | d) Ulcer |

Q.146) A 38-year-old man presents to the emergency department with sudden onset severe flank pain radiating to the groin. The pain is colicky, intermittent, and associated with nausea and restlessness. He reports difficulty in passing urine and occasional hematuria. On examination, there is tenderness in the costovertebral angle. Laboratory tests reveal microscopic hematuria. An X-ray KUB shows a well-defined radio-opaque shadow in the region of the kidney. What is the most likely diagnosis?

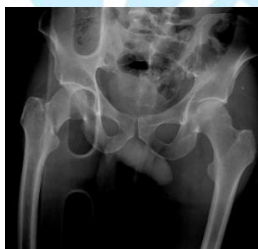


- | | | | |
|----|-------------|----|-------|
| a) | Gallstone | c) | Tumor |
| b) | Renal stone | d) | Cyst |

Q.147) A 72-year-old man with a long history of hypertension, smoking, and hyperlipidemia presents for evaluation of vague, dull abdominal discomfort radiating to the back. On examination He is hemodynamically stable. Based on the CT finding of abdominal aorta, which of the following is the most likely diagnosis?



- | | | | |
|----|---------------------------|----|-------------------|
| a) | Abdominal aortic aneurysm | c) | Aortic stenosis |
| b) | Aortic dissection | d) | Aortic thrombosis |



Q.148)

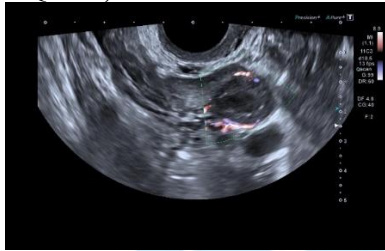
Complication of the above condition ?

Complication?

- a) AVN
b) Arthritis only

- c) Myopathy
d) Infection

Q.149)



- a) Ectopic pregnancy
b) Corpus luteum cyst

- c) Fibroid
d) Ovarian torsion

Q.150)



A chest X-ray shows complete opacification of the right hemithorax. The trachea and mediastinum are shifted to the left.

- a) Massive pleural effusion
b) Total lung atelectasis

- c) Pneumonia
d) Pulmonary fibrosis

ANSWERS

1	b
2	a
3	b
4	b
5	b
6	a
7	a
8	a
9	c
10	a
11	a
12	a

13	a
14	b
15	b
16	b
17	b
18	b
19	b
20	b
21	b
22	a
23	a
24	b
25	b

26	b
27	a
28	b
29	b
30	a
31	b
32	c
33	b
34	b
35	b
36	a
37	a
38	a

39	a
40	a
41	a
42	a
43	b
44	a
45	a
46	a
47	a
48	a
49	a
50	a
51	a

52	a
53	a
54	a
55	a
56	a
57	a
58	b
59	b
60	b
61	a
62	b
63	c
64	a



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65	a
66	b
67	b
68	b
69	b
70	b
71	b
72	b
73	a
74	a
75	a
76	b
77	b
78	b
79	a
80	b
81	a
82	b

83	b
84	b
85	a
86	b
87	b
88	c
89	c
90	c
91	c
92	c
93	d
94	c
95	d
96	a
97	a
98	c
99	b
100	c

101	b
102	b
103	a
104	b
105	c
106	b
107	b
108	a
109	b
110	b
111	b
112	b
113	b
114	b
115	b
116	b
117	b
118	b

119	b
120	a
121	b
122	b
123	a
124	a
125	b
126	c
127	b
128	c
129	c
130	a
131	b
132	b
133	b
134	b
135	a
136	a

137	b
138	a
139	b
140	c
141	b
142	b
143	b
144	a
145	a
146	b
147	a
148	a
149	a
150	a